

**Veterans' Health Administration (VHA), Office of Women's Health (OWH)
Mini-Residency for Rural Providers and Nurses Training Program (Physician Instructors,
Nurse Instructors, Gynecological Teaching Associates and Equipment Support)
Performance Work Statement (PWS)**

1. BACKGROUND

The Veterans Health Administration (VHA) patient population of women Veterans is growing rapidly. Such growth brings increasing demand for high quality women-specific health care. Approximately 27% of enrolled women Veterans reside in rural and highly rural areas of the county; areas in need of more trained primary care providers who are also designated as Women's Health Primary Providers (WH-PCP) as well as nursing staff trained in women's health.

Office of Women's Health (OWH) has developed and delivered a comprehensive education and training model called the Women's Health Mini-Residency since 2008 for clinical staff to address gaps in knowledge and skills in women's health topics. Although over 14,000 primary care providers and primary care nurses have been trained in a mini-residency program, additional training needs persist, particularly in rural, highly rural, and insular-island areas. Thus, a Women's Health Mini-Residency for Rural Providers and Nurses was specifically designed for primary care clinicians in rural areas.

The mini-residency training program described in this PWS, in conjunction with prerequisite online content for a blended learning approach, is intended to improve familiarity, knowledge and skills of rural primary care providers and nurses in women's health topics, thereby increasing the number of and access to WH-PCPs and nurses trained in women's health in rural areas and ultimately enhancing and improving the high-quality healthcare women Veterans receive.

2. ACRONYMS

ACGME – Accreditation Council for Graduate Medical Education
AQL – Acceptable Quality Level
AV – Audio/Visual
C&A – Certification and Accreditation
CBOC – Community Based Outpatient Clinic
CO – Contracting Officer
COR – Contracting Officer's Representative
CV – Curriculum Vitae
ESI – Equipment Support Individual
FAR – Federal Acquisition Regulation
FTR – Federal Travel Regulation
GFE – Government Furnished Equipment
GFI – Government Furnished Information
GTA – Gynecological Teaching Associate
ILEAD – Institute for Learning, Education and Development
IPV – Intimate Partner Violence
LPN – Licensed Practical Nurse
MOP – Management Oversight Plan
OWH – Office of Women's Health
PAP – Performance Assessment Plan
PCP – Primary Care Provider
PWS – Performance Work Statement
QASP – Quality Assurance Surveillance Plan
PRS – Performance Requirements Summary

RN – Registered Nurse
TTT – Train the Trainer
U.S. – United States
VA – Department of Veterans Affairs
VHA – Veterans Health Administration
WHMD – Women’s Health Medical Director
WVPM – Women Veterans Program Manager

3. SCOPE OF WORK

The Office of Women’s Health (OWH) within Veterans Health Administration (VHA) seeks to obtain a contract with a company with current staff positions of clinical instructors, gynecological teaching associates (GTA) and equipment support individuals (ESI) with experience in the instruction and delivery of VHA Women’s Health subject matter. The Contractor shall provide a trained team (one (1) physician instructor, one (1) nurse instructor, one (1) gynecological teaching associate (GTA), and one (1) equipment support individual (ESI)) with experience in the delivery of OWH-developed interprofessional Women’s Health Mini-Residency training to cohorts of 2–12 clinical learners at up to 40 rural VHA clinical sites across the U.S. and its territories within a 12-month period. Training includes small-group case facilitation, hands-on clinical simulation, team-based process improvement, and structured participant feedback.

Contingency Delivery: When in-person delivery is impacted (e.g., pandemics, disasters), Contractor shall deliver designated portions virtually per Section 5.6 and complete remaining hands-on components on site when allowable.

The Contractor will ensure ESI staff will have an onsite presence for one-half day the day prior to the training at a time determined by the onsite point of contact to assist with training space, equipment, and supply set-up. All staff, except for the GTA, will be onsite on the day of the training from 7:30 am (local time) through the training event’s conclusion at 6:00 pm (local time) providing the ESI an extra hour for packing of equipment. The Contractor will ensure GTA is onsite no later than 12:00 pm to check in with the instructor teams and/or ESI before pre-brief of hands-on activities begins. The Contractor shall ensure GTA remains onsite to fulfill tasks until program conclusion at 5:00 pm (local time) on the day of training. The Contractor shall provide alternate or back-up physician instructor, nurse instructor, GTA and ESI coverage to ensure 100% attendance of each at each training event. Note: Hours listed above may change according to training event delivery method due to pandemics or other special circumstances but never exceed the number of hours listed above.

The Contractor shall ensure they have the appropriate personnel to provide the requirements prescribed by this contract. A list of personnel meeting the experience criteria below will be provided to VA upon contract award and when additional personnel are added to Contractor staff. Resume for each staff member will be provided to OWH to confirm experience requirements are met. OWH will provide concurrence of personnel in writing via email. Once the Contractor makes their final choices of required staff, Contractor will coordinate with OWH to provide a train-the-trainer session.

4. PERFORMANCE DETAILS

4.1. TYPE OF CONTRACT

The Government anticipates award of a Firm Fixed-Price contract.

4.2. Period of Performance. The period of performance for this contract shall be one (1) year from date of award.

4.3. Performance Incentives. **The Government will closely monitor the Contractor’s** performance in meeting the performance objectives and standards set forth in this contract. Based on the Government’s assessment of the Contractor’s performance and the availability of funding, the

Government may elect to exercise contract options to extend the period of performance.

4.4. **Place of Performance.** Pre-requisite preparation and review of training content will be performed at Contractor's location. The "train-the-trainer" workshop will be held at a site to be determined and communicated by OWH. All training events will be held at VHA clinical sites within the U.S. and U.S. Territories. Multiple training events may occur in the same week depending on training locations.

4.5. **Observance of Government Holidays.** There are 11 Federal holidays set by law (USC Title 5 Section 6103). Under current definitions, five are set by date:

New Year's Day	January 1
Juneteenth National Independence Day	June 20
Independence Day	July 4
Veterans Day	November 11
Christmas Day	December 25

If any of the above falls on a Saturday, then Friday shall be observed as a holiday. Similarly, if one falls on a Sunday, then Monday shall be observed as a holiday. The other six holidays are set by a day of the week and month:

Martin Luther King's Birthday	Third Monday in January
Washington's Birthday	Third Monday in February
Memorial Day	Last Monday in May
Labor Day	First Monday in September
Columbus Day	Second Monday in October
Thanksgiving	Fourth Thursday in November

4.6. **Training Changes or Cancellations**

The Government will generally only cancel training due to:

- Severe emergencies, weather conditions, pandemics or critical national events that are unforeseen and unpredictable; and
- During Federal-wide or agency shut down periods.
- Training may also be cancelled due to an occasional need to change the dates and locations of training. The Government will notify the contractor when training dates are changed or cancelled due to any of these circumstances.
- When the contractor is given a minimum of 21 calendar days' notice prior to the scheduled training start date the Government may cancel and/or substitute, relocate, or reschedule training without penalty and at no additional cost to the Government.
- When less than 21 calendar days' notice is given (e.g., in the event of an emergency or Government shut down), training may be substituted, relocated, or rescheduled by mutual agreement of the Government and contractor, within the task order period of performance, at no additional cost to the Government.
- If the Government does not provide notice within 21 calendar days and the training is not rescheduled, the contractor will receive a fee of 20% of the scheduled training cost.

4.7. **TRAVEL**

Travel will be required for the "train-the-trainer" workshop to a site to be determined by OWH. Travel will be required for all training events for up to 40 VHA clinic sites in the U.S., its commonwealths and territories, over a twelve (12) month period of performance for a total of one (1) year. Multiple training events may occur in the same week depending on training locations: therefore, travel is anticipated between VHA facilities and Community Based Outpatient Clinics, which may result in overnight stays and reasonable driving distances. There is up to 100% travel

required with the potential of extending over weekends on occasions, including the normal everyday risks and discomforts of auto, bus, rail, and air transportation. Training sites will be identified by OWH and provided to the Contractor no later than 45 days prior to the event training. Contractor shall ensure staff have appropriate identification for travel.

Travel must be pre-approved by the Contracting Officer's Representative (COR) and will be reimbursed in accordance with Federal Travel Regulations and Federal Acquisition Regulations (FAR) 31.205-46. Each contractor's invoice must include copies of all receipts that support the travel costs claimed in the invoice. Travel performed for personal convenience and daily travel to and from work at the Contractor's facility will not be reimbursed. The COR will document his/her pre-approval in writing. The Contractor and subcontractors deviating from this may have certain travel expenses declined.

5. TASKS AND PERFORMANCE OBJECTIVES

The contractor shall perform the following tasks in accordance with U.S. Government Public Laws, policy directives, regulations, and non-Government standards.

5.1. Conduct a "kick-off" meeting within three (3) business days of award.
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The kick-off meeting will consist of the contractor participating in an online conferencing orientation for program overview and introduction to Government Furnished Information (GFI) used in the training event. Background information, training event agenda, and GFI will be presented. Physician/nurse instructors, ESIs and GTAs may attend at the contractor's discretion.

Deliverable: Schedule kick-off within 3 business days of award of contract.

5.2. Attend, manage, and document monthly meetings with OWH.
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Deliverables: The Contractor shall provide meeting minutes, monthly progress, status, and management reports informing of the progress of each task, identifying accomplishment, issues, concerns, and proposed resolutions. The monthly status report will specifically include an itemized list of tasks completed, the hours, amounts to be charged for each task and all other costs associated for that billing period.

All documents presented will be concurred upon by the Contracting Officer's Representative (COR) within 10 days of receipt and prior to invoicing by contractor for that billing period.

5.3. Ensure staff has met the appropriate prerequisites to perform their duties.
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5.3.1. Physician and nurse Instructors.

The Contractor shall ensure that:

- 5.3.1.1. Physician instructors view OWH-developed online lectures that are pre-requisites for VHA providers learners. Time required to complete the viewings is estimated to take 10.5 hours. Access to these GFI items will be provided to the Contractor by OWH.
- 5.3.1.2. Nurse instructors view WH-developed online lectures that are pre-requisites for VHA nurse learners. Time required to complete the viewings is estimated to take 10 hours. Access to these GFI items will be provided to the Contractor by OWH.
- 5.3.1.3. Physician and nurse instructors master all content comprising the training event to the necessary extent to facilitate discussion and instruct hands- on learning. Time required to master all content comprising the training is estimated to take 28 hours. Access to these GFI items will be provided to the Contractor by OWH.

- 5.3.1.4. Physician and nurse instructors study evaluation tools used to evaluate instructors and the training. Time required to study these GFI items is estimated to take one (1) hour. Access to these GFI items will be provided to the Contractor by OWH.
- 5.3.1.5. Documentation is provided to confirm physician and nurse instructors have viewed and mastered all content comprising the training event to the necessary extent to deliver, facilitate discussion and instruct hands-on learning. This documentation must be provided to OWH for each instructor five (5) days prior to attending a “train-the-trainer” workshop.
- 5.3.1.6. Physician and nurse instructors who will be delivering the training event must attend a “train-the-trainer” workshop. This “train-the-trainer” workshop may be held up to twice annually and could include up to two (2) physician and nurse instructors in each year of the contract. This “train-the-trainer” workshop will be organized and delivered by OWH-identified instructors. This workshop will allow for 1) diffusion of OWH standard training structure and content; 2) opportunity for questions and clarifications regarding content and means of delivery; 3) practice with simulation equipment and supplies; and 4) real-time logistical practice with training equipment and supply set-up and break-down. Anticipated time requirement for the “train-the-trainer” workshop is estimated at 24 hours (three days (3) 8-hour days), not including travel time, and will be held at a facility to be determined and provided by the Government. Contractor shall provide and support travel for physician and nurse instructors to this “train-the-trainer” workshop.

Deliverables: Documentation of each instructor viewing and mastering all content and attendance of face-to-face “train- the-trainer” training.

5.3.2. GTA.

The Contractor shall ensure:

- 5.3.2.1. GTAs view one (1) VHA breast exam video and two (2) VHA pelvic exam videos (routine and acute) developed by OWH. Time required to complete the viewings is estimated to take 1 (one) hour. Access to these GFI items will be provided to the Contractor by OWH.
- 5.3.2.2. GTAs master all content related to the GTA activity comprising the training event to the necessary extent to facilitate the GTA activity, instruct on breast and pelvic examination technique and provide participant feedback. Content includes training agenda, case scenarios, examination checklists, pre-brief and de- brief templates. Time required to master all content related to the GTA activity is estimated to take three (3) hours. Access to these GFI items will be provided to the Contractor by OWH.
- 5.3.2.3. GTAs study evaluation tools used to evaluate the GTA and the training. Time required to study these GFI items is estimated to take one (1) hour. Access to these GFI items will be provided to the Contractor by OWH.
- 5.3.2.4. Documentation is provided to confirm GTAs have viewed and mastered all content comprising the training event to the necessary extent to facilitate the GTA activity, instruct on breast and pelvic examination technique and provide participant feedback. This documentation must be provided to OWH for each instructor five (5) days prior to attending a “train-the-trainer” workshop.
- 5.3.2.5. GTAs attend a face-to-face “train-the-trainer” workshop in advance of their first training event. All GTAs who will be providing GTA services for the training must attend a “train-the-trainer” workshop. This “train-the-trainer” workshop may be held up to twice annually and could include up to five (5) GTAs. This “train-the-trainer” workshop will be organized and delivered by OWH-identified instructors. This workshop will allow for 1) diffusion of OWH standard training structure and content; 2) opportunity for questions and clarifications regarding content and means of delivery and; 3) real-time logistical practice with training flow. Anticipated time requirement for the “train-the-trainer” workshop for GTAs estimated is one

day (eight (8) hours), not including travel time, and will be held at a facility to be determined and provided by Government. Contractor shall provide and support travel for GTAs to this “train-the-trainer” workshop.

Deliverables: Documentation of each GTA viewing and mastering all content and attendance of face-to-face “train-the-trainer” training.

5.3.3. ESI.

The contractor shall ensure that:

- 5.3.3.1. ESIs review OWH-developed content comprising the training curricula (GFI) relevant to the use of simulation of the GFE and hands-on learning. Access to these GFI items will be provided to the Contractor by OWH. Time required to review content relevant to the use of simulation of the GFE and hands-on learning is estimated to take five (5) hours. All said equipment, materials and items will be provided to the Contractor by OWH.
- 5.3.3.2. Documentation is provided to confirm the ESI has reviewed all content relevant to simulation and hands-on learning comprising the training event. This documentation must be provided to OWH for the ESI five (5) days prior to attending a “train-the-trainer” workshop.
- 5.3.3.3. ESIs attend a face-to-face “train-the-trainer” workshop in advance of their first training event. This “train-the-trainer” workshop may be held up to twice annually and could include up to two (2) ESI’s in in each year of the contract. This “train-the-trainer” workshop will be organized and delivered by WH- identified instructors. This workshop will allow for 1) diffusion of OWH standard training structure and content; 2) opportunity for questions and clarifications regarding content and means of delivery; 3) practice with simulation equipment and supplies; and 4) real-time logistical practice with training equipment, other hands-on activities and supply unpacking, set-up, break-down and packing. Anticipated time requirement for the “train-the-trainer” workshop is estimated at 20 hours (two and one-half days (2.5) 8-hour days), not including travel time, and will be held at a facility to be determined and provided by Government. Contractor shall provide and support travel for the ESI to this “train-the-trainer” workshop.

Deliverables: Documentation of review of all content required with emphasis on content for simulation and hands on learning and attendance at the face-to-face “train-the-trainer” workshop.

- 5.3.3.4. In the event where training hours are not utilized, OWH may utilize those hours for instructors, GTAs and ESIs to master updated training content, observe training events, and/or participate in the delivery of train-the-trainer workshops for new, onboarding contract instructors, GTAs and ESIs.

5.4. Contractor will ensure staff provides onsite duties as prescribed.

5.4.1. Instructors.

The Contractor shall ensure that:

- 5.4.1.1. The physician and nurse instructors co-present a welcome, introduction and overview of training OWH-developed slide deck/GFI at the beginning of the training event. This provides background on women Veterans in VHA and an overview of the training event to participants. This also includes introduction of a VHA Women Veteran’s Program Manager (WVPM) and/or Women’s Health Medical Director (WHMD) to participants and would be arranged by OWH in advance of the training event. Instructors are responsible for assisting the ESI in ensuring all participants complete a sign-in sheet during the welcome to confirm their presence at the training.

- 5.4.1.2. The physician and nurse instructors co-facilitate five (5) small group case discussions using OWH-developed slide decks/GFI. Co-facilitation must be done in a manner that encourages interprofessional dialogue, team learning and interaction by all participants. Case discussions are patient scenarios and serve to reinforce and apply material covered in online lectures. The Contractor will also introduce and incorporate VHA staff in discussions regarding special content such as Intimate Partner Violence (IPV). The VHA staff will be arranged by OWH and communicated to the Contractor in advance of the training event.
- Physician and nurse instructors instruct participants on an approach to organizational change and facilitate completion of a process improvement project (action plan) using OWH-developed supporting slides and a template for 'action plan'. Throughout the training event, physician and nurse instructor will help participants identify a gap/issue in the care of women Veterans at their clinical site that may be addressed via the 'action plan'. This may be done in teams or individually. Time will be allotted near the end of the training for completion of the template. Contractor shall ensure that duplicate copies of the completed action plans are collected by instructors and provided to OWH within five (5) business days of post-training.
- 5.4.1.3. The physician and nurse instructors co-facilitate a pre-brief to simulation activities (handout) that outlines the rules of trust, roles and responsibilities, and logistics of the simulation stations using a pre-brief outline as provided by OWH. The pre-brief will include an introduction of the GTA while dressed in street clothes. The instructors will reinforce that professionalism is always expected and encourage use of breast and pelvic exam checklists.
- 5.4.1.4. The Physician instructors explain, demonstrate, and instruct the proper standardized technique endorsed by VHA for breast and pelvic examination. This technique should be consistent with what is included in the breast and pelvic examination videos, detailed in the breast and pelvic exam checklists, and explained in the simulation equipment "table talk". Use of breast and pelvic exam simulation equipment/task trainers will be required and will be set-up in station, rotation format. Physician instructors should assist participants on identifying abnormalities on pelvic exam simulation equipment / task trainers and describing the findings. Feedback to providers' participants on their exam technique and identification of abnormalities in a manner that is collegial, informative, and constructive is expected.
- 5.4.1.5. Nurse instructors explain, demonstrate, and instruct the proper history taking/triage questioning, exam room preparation, and breast and pelvic exam set-up and assistance for various women's health presentations. Scenario-based hands-on activities using breast and pelvic exam checklists, suggested women's health questions and pelvic exam scenario will be required and will be set-up in a station, rotation format. Feedback to nurse participants on their line of questioning, appropriate language used, and identification of necessary supplies in a manner that is collegial, informative, and constructive is expected.
- The physician and nurse instructors explain, demonstrate, and instruct on appropriate language and professional and respectful behavior to use when preparing for, assisting with, and performing breast and pelvic exam that follows VHA guidance, maintains patient respect, prioritizes patient comfort and dignity, and considers a patient's history of trauma. GFI that includes chaperone requirements, patient draping and room set-up for breast and pelvic exam must be followed.
- 5.4.1.6. The physician and nurse instructors are available to assist with instruction clarification of history and exam techniques when requested for the GTA live breast and pelvic exam activity when requested by participants or staff. While this activity is intended to take place without direct physician and /or nurse instructor involvement, the instructors may be called upon in the event their assistance is required with the GTA activity. Involvement may include, but not limited to,

- assistance with room, supplies, equipment, or aspects of breast and/or pelvic exam.
- 5.4.1.7. The Physician and nurse instructors explain, demonstrate, and instruct on use of over 70 item gynecologic equipment and supplies ("show-and-tell" table). A comprehensive list of all items that include details on indications and usage instructions for each item will be provided as GFI.
 - 5.4.1.8. Physician and nurse instructors interact with the MammaCare breast exam haptic simulator and participants when necessary. This activity is intended for provider participant use but may be utilized by nurse participant if time permits. While this activity is intended to take place without direct physician and/or nurse instructor involvement, the instructors may be called upon in the event assistance is required for the MammaCare breast exam haptic simulator activity. Involvement may include, but not limited to, breast exam technique or technical troubleshooting.
 - 5.4.1.9. The physician and nurse instructors support the gynecologic procedure videos with associated patient education activity. This activity is intended for nurse participant use but may be utilized by provider participants if time permits. While this activity is intended to take place without direct physician and/or nurse instructor involvement, the instructors may be called upon in the event assistance is required for the gynecologic procedure videos and associated patient education activity. Involvement may include, but not limited to, fielding questions about gynecologic procedures viewed or technical troubleshooting.
 - 5.4.1.10. The physician and nurse instructors co-facilitate a debrief session of simulation activities (using a GFI handout) that allows for an open discussion about each simulation activity, including the GTA activity. The debrief will include summative feedback to the group and/or concluding comments from the GTA while dressed in street clothes.
 - 5.4.1.11. The physician and nurse instructors provide participants with OWH-provided GFI to include, but not limited to, resources list, learning opportunities, and education handouts. A list of additional resources and websites provided as GFI may be accessed during training events depending on the participant's learning needs.
 - 5.4.1.12. On a case-by-case basis, physician and/or nurse instructors are available to assist the ESI with the training space, simulation equipment and supplies to support the training event if needed and cleared by OWH prior to assigning. This may include assistance with room arrangement of the physical training space and set-up of said equipment and supplies. These tasks may take place one-half day prior to the training event. This may require lifting items no heavier than 100 pounds.
 - 5.4.1.13. The physician and nurse instructors adhere to the training event schedule agenda as close as possible, maintaining time allotments for each discussion or activity. Attention to timekeeping and the holding of participants accountable to the agenda will be required. The training event shall begin promptly at 8:00 am, must not conclude later than 5:00 pm and must include a 30-minute lunch and two (2) 15-minute breaks for participants (accounted for in the training event agenda). The ESI may assist with onsite timekeeping.

5.4.2.GTA.

The Contractor shall ensure that:

- 5.4.2.1. GTAs participate in instructor-led pre-brief to hands-on activities, including an introduction of the GTA to participants prior to GTA activity. This will allow for introduction of GTA while in street clothes prior to beginning of GTA activity.
- 5.4.2.2. GTAs provide guidance to each nurse participant on the following tasks: 1) appropriate assessment / history-taking for a woman presenting for breast and pelvic exam using OWH-developed scenarios, 2) exam room and supply set-up, 3) language used/communication skills, and 4) nursing-focused care before, during and

after breast and pelvic examination. In this nurse participant only session of the GTA activity, the GTA will function primarily in a standardized patient role, portraying a patient scenario. The Contractor shall ensure guidance during the GTA activity aligns with contents of GFI and videos, is provided in a manner that enhances, but not interferes with the training experience, and adheres to time- allotment in the training event schedule.

- 5.4.2.3. GTAs provide guidance to each provider participant on the following tasks: 1) appropriate supplemental history-taking for a woman presenting for breast and pelvic exam using OWH-developed scenarios, 2) language used/communication skills, and 3) proper technique to perform breast and pelvic examination. Contractor shall ensure guidance during the GTA activity aligns with contents of GFI and videos, is provided in a manner that enhances, but not interferes with the training experience, and adheres to time-allotment in the training event schedule.
- 5.4.2.4. GTAs participate, for each training event, in up to four (4) separate sessions with a nurse performing a patient assessment / history-taking based on up to two (2) different pre-determined brief case scenarios provided as GFI. This component requires the GTA to function more in a standardized patient role portraying a patient scenario, rather than a GTA role, although GTA will be undressed and gowned during this interaction. Registered nurse (RN) participants will practice in-person triage skills with the GTA acting as a female patient presenting with a chief complaint of vaginal bleeding and will be given results indicating normal vital signs and negative pregnancy test results. In this scenario, the GTA will also request a breast exam. Licensed practical nurse (LPN) participants will practice history-taking skills with the GTA acting as a female patient presenting for breast exam and cervical cancer screening and will be given results indicting normal vital signs. Contractor shall ensure GTA provide fictional responses to the nurse questioning. Contractor shall ensure GTA allows nurse participants an opportunity to attend to room and exam supply set-up at the conclusion of the encounter in anticipation of breast and pelvic examination by the provider. Nurse participants may use GFI breast and pelvic exam checklists to aid in these tasks.
- 5.4.2.5. GTAs participate in up to four (4) separate sessions with a provider conducting a complete breast and pelvic examination with a 'mock' Pap test sample collection. This component will be done with a provider-nurse team working together simulating a real clinical setting. After the nurse participant briefly presents the patient assessment or history to the provider outside of the exam room, the provider-nurse team will enter the GTA room together. Nurses will act as a chaperone and assist the provider during the exam process and support the GTA as a female patient. Providers may practice obtaining additional history-taking for clarifying information in addition to what the nurse presented. Contractor shall ensure GTA provide fictional responses to the provider questioning. Providers will conduct breast examinations to include visual inspection, palpation in axillary and supraclavicular areas, and bilateral breast palpation. Providers will conduct pelvic examination to include external inspection, speculum examination, bimanual examination, and 'mock' Pap test sample collection (actual specimen collection will not be performed). Provider participants may use GFI breast and pelvic exam checklists to aid in these tasks.
- 5.4.2.6. GTAs provide each participant with summative, qualitative feedback by individual provider participant and nurse participant as well as by provider-nurse team, based on GFI and videos, to align with VHA-endorsed best practices. The Contractor shall ensure GTA provides qualitative feedback to participants, as a minimum, on the use of appropriate language/communication skills, thoroughness of assessment/history, appropriate room/exam set-up, performance of breast and pelvic examinations and team interaction. GTA qualitative feedback

shall be provided verbally to participants immediately at the conclusion of the GTA activity for each provider-nurse team. The GTA shall provide nurse and/or provider participants with the opportunity to repeat all or portions of the assessment/history, breast and/or pelvic exams during the session after feedback is provided to reaffirm what was learned during the GTA activity.

- 5.4.2.7. VHA clinical faculty and/or contract instructors are able to directly observe interactions between the GTA and the participant(s) in the examination room. This may be necessary depending on the number of nurse participants in the training or in the event additional assistance is required. GTA privacy and dignity will be always attended to during GTA activity.
- 5.4.2.8. GTAs participate in an instructor-led debrief and training event conclusion at the end of each training event. This would be an opportunity for GTA to provide general feedback to the provider and nurse participants as a group, as necessary and time permitting. This may include tasks that were done well among several participants or areas in need of attention or skill-building among several participants. Contractor shall ensure GTA refrains in talking about specific participants' actions or techniques in the group setting.
- 5.4.2.9. GTAs adhere to the training event schedule agenda as close as possible, maintaining time allotments for each GTA activity. Attention to timekeeping and holding participants accountable to the agenda will be required. The training event must not conclude later than 5:00 pm. The equipment and set-up support individuals may assist with onsite timekeeping.

Deliverables: Documentation of instructing and facilitating all content comprising the training event, adhering to training schedule.

5.4.3. Equipment Support Individual (ESI).

The contractor shall ensure that:

- 5.4.3.1. ESIs receive simulation equipment, training supplies and consumable items from a VHA facility or training site in proximity to a training event site and ensure said GFE is transported and accounted for at each transition. Preparing GFE for vehicle transport will be necessary. There is a requirement to drive a vehicle of adequate size (i.e., minivan) to comfortably fit all GFE (approximately 48-cubic feet) for transporting equipment between clinic sites. The contractor will be responsible for the cost of obtaining the vehicle. Additionally, physical activity such as lifting moderately heavy objects (e.g., boxes and shipping cases up to 100 pounds in weight) will be required.
- 5.4.3.2. ESIs prepare, manage, and maintain simulation equipment, training supplies (including media and contractor-supplied computers), consumable items used during the training events at each clinic site. This includes packing, unpacking, set-up, break-down, cleaning, and monitoring utilization of GFE. Assistance with training space set-up and limited moving of furniture may also be necessary. Contract instructors may assist with on-site set-up, preparation, break- down, packing and training space set-up.
- 5.4.3.3. ESIs provide administrative and technical support to OWH, the contracted training team (physician and nurse instructors) and clinic sites point of contact as needed to support the training event and simulation activities. This includes, but is not limited to, audiovisual support, securing additional supplies in real-time, printing, copying, timekeeping, monitoring of simulation activities, participant sign in (to be completed during the welcome) and tracking. The Contractor shall ensure the ESI provide the completed sign-in sheet to OWH by 10:00 am local time on the training day.

- 5.4.3.4. ESIs assist participant and contract training team (physician and nurse instructors) with simulation work, the use of simulation equipment and serves as a resource to OWH and contract training team (physician and nurse instructors) utilizing simulation. The level of assistance may vary and may take place at the 'train-the-trainers' workshop, onsite at the training events and/or other times during the contract.
- 5.4.3.5. ESIs assist participants and the GTA with the GTA activity, reinforcing tasks and expectations of participants, timekeeping, monitoring exam room door and serving as a resource for the GTA.
- 5.4.3.6. ESIs operate and maintain all AV equipment used in the training event. This includes, but not limited to, set-up, connectivity, projection, and technical troubleshooting of contractor-supplied laptop computers as well as GFE accessories, MammaCare breast exam haptic simulator and other AV equipment.
- 5.4.3.7. Proper utilization of GFE to support the training event to include breast simulation equipment, pelvic simulation equipment, 70 gynecologic items ("show-and-tell" items), consumable items for breast and pelvic exam activities, and ancillary items such as power cords and portable tables. Usage of GFE will be in accordance with manufacturer recommendations and follow OWH guidance.
- 5.4.3.8. Provision of two (2) Contractor-furnished laptop computers, both with speaker and headphone capabilities, and include necessary connections, ports, and/or adapters that allow connection to a larger projection screen (i.e., USB, HDMI) at each training event. Laptop computers will be used for instructor projected display of training materials, group and individual viewing of videos and facilitation of patient education activity during simulation activities.
- 5.4.3.9. ESIs ensures security and safe keeping of all simulation equipment, training supplies, consumable items and AV equipment at each training event sites and during transport/transition times. If equipment should be lost, damaged or stolen while in transport and/or in possession of the ESI, the contractor will be responsible for the replacement of said equipment.

Deliverables: Documentation of ESI receiving, transporting, managing, inventorying, and shipping GFE.

5.5. Contractor will ensure instructors provide feedback post-training to assist in the improvement, evaluation, and monitoring of the training program

5.5.1. Instructors. The Contractor shall ensure that:

- 5.5.1.1. The physician and nurse instructors notify ESI of damage/replacement or additional equipment needs and work with the ESI to evaluate ongoing supply and equipment needs after each training.
- 5.5.1.2. The physician and nurse instructors provide a written post-training report to OWH using an OWH-developed reporting template within five (5) business days after the conclusion of each training event. Post-training report should include feedback about training day logistics and recommendations to improve the training and may not include any personally identifiable information about participants. If serious or significant issues occur during the training event, they must be reported immediately to the site POC (name provided by OWH) and OWH. Copies of Sign-in sheets and participant-developed action plans must accompany the written post-training report.
- 5.5.1.3. The physician and nurse instructors participate in monthly post-training feedback sessions during months of any training event with OWH via virtual conferencing. Each post-training after action feedback session will last approximately 1 – 1.5 hours.

OWH will be responsible for scheduling and coordination of feedback sessions. These feedback sessions will allow for a more in-depth discussion on the training event structure, content, and logistics.

5.5.2. GTA. The Contractor shall ensure that:

- 5.5.2.1. GTAs provides a written post-training report to OWH using an OWH-developed reporting template within five (5) business days after the conclusion of each training event. Post-training report should include GTA feedback about training day logistics and recommendations to improve the GTA activity but may not include any personally identifiable information about participants.
- 5.5.2.2. GTAs participate in monthly post-training feedback sessions during months of any training event with OWH via virtual conferencing. Each post- training after action feedback session will last approximately 1 -1.5 hours. OWH will be responsible for scheduling and coordination of feedback sessions. These feedback sessions will allow for a more in-depth discussion on the training event structure, content, and logistics.

Deliverables: Post-training report and documentation of post training after action feedback sessions.

5.5.3. ESI. The Contractor shall ensure:

- 5.5.3.1. The ESI inventories, cleans, packs, and prepares all GFE for transport to another training event site and/or shipping to a central storage location or VHA facility within two (2) hours of the conclusion of specified training events as directed by OWH. This may include transport of GFE within a VHA facility for shipping or other shipping centers. The contractor will be responsible for the cost of obtaining the vehicle. Shipping costs will be covered by OWH. The ESI must provide an inventory list of GFE post-training for each training event that accompanies GFE upon return to a central storage location.
- 5.5.3.2. The ESI ship simulation equipment, training supplies and consumable items to a central storage location after the conclusion of identified training, using shipping mechanism provided by OWH. A GFE Receiving report will be completed by central storage location point of contact and provided to OWH within five (5) days of GFE receipt.
- 5.5.3.3. There is notification to OWH of damaged/replacement or additional equipment needs and work with the physician and nurse instructors to evaluate ongoing supply and equipment needs after each training. Completion of a written OWH Post-training report will be required after each training event and provided to OWH within five (5) business days after the conclusion of each training event.
- 5.5.3.4. The ESI collaborate in maintenance and repair of clinical simulation equipment as necessary.
- 5.5.3.5. The ESI participates in monthly post-training feedback sessions during months of any training event with OWH via virtual conferencing. Each post- training after action feedback session will last approximately 1-1.5 hours. OWH will be responsible for scheduling and coordination of feedback sessions. These feedback sessions will allow for a more in-depth discussion on the training event structure, content, equipment, and logistics.

Deliverables: Post-training report, inventory list of GFE, and documentation of post training after action feedback sessions.

5.6. In cases of pandemics, natural disasters, or emergencies where restrictions on travel and/or face-to-face training exist, the program will be delivered by the Contractor in two phases

Phase 1) virtually for the welcome, introduction, and case discussions to include section 5.5 which require the physician and nurse instructors only; and
Phase 2) in-person for the hands-on simulation and GTA sessions, 5.4.1.3 through 5.4.1.15. 5.4.2, 5.4.3 and 5.5, as prescribed under the current program and when allowable per guidance of VA, except for the start and end times which will be determined and communicated to the contracting staff in advance of the training events. The Contractor will ensure instructors provide virtual duties as prescribed.

5.6.1. Instructors. The Contractor shall:

- 5.6.1.1. Identify physician and nurse instructors to attend 1-2 planning meetings (up to 2 hours each) to assist in recommendations to deliver the program virtually. This may include practice sessions for the virtual training.
- 5.6.1.2. Ensure the physician and nurse instructors have the technical capabilities and equipment to provide the best quality presentation to the participants. This will include strong internet connection, clear audio, and video capabilities. The virtual platform will be provided and/or hosted by VA's Institute for Learning, Education and Development (ILEAD) and/or OWH.
- 5.6.1.3. Ensure the physician and nurse instructors are in an environment that is quiet and free from distractions when providing virtual training.
- 5.6.1.4. Ensure the physician and nurse instructors co-present a welcome, introduction and overview of training OWH-developed slide deck/GFI at the beginning of the training event. This provides background on women Veterans in VHA and an overview of the training event to participants. This also includes introduction of a VHA Women Veteran's Program Manager (WVPM) and/or Women's Health Medical Director (WHMD) to participants that may be done virtually (audio and/or video) and would be arranged by OWH in advance of the training event. Instructors are responsible for confirming that all participants are present at the training; a list of the participants will be provided electronically by OWH to confirm

attendance. The completed attendance list will be due to OWH by 10:00 am local time on the training day.

- 5.6.1.5. Ensure physician and nurse instructor instruct participants on an approach to organizational change and a process improvement project (action plan) using OWH-developed supporting slides. Throughout the training event, physician and nurse instructor will help participants identify a gap/issue in the care of women Veterans at their clinical site that may be addressed via the 'action plan'. Time will be allotted near the end of the phase 2 in-person training for completion of the template.
- 5.6.1.6. Ensure the physician and nurse instructor co-facilitate five (5) small group case discussions virtually using OWH-developed slide decks/GFI. Virtual co-facilitation must be done in a manner that encourages interprofessional dialogue, team learning and interaction by all participants. Case discussions are case-based and serve to reinforce and apply material covered in online lectures. This also includes introduction/incorporation of a VHA Intimate Partner Violence (IPV) Coordinator to participants that may be done virtually (audio and/or video) and would be arranged by OWH in advance of the training event.
- 5.6.1.7. Ensure the physician and nurse instructor conclude the virtual training by asking participants to reflect upon and share 'take-home points' from the discussion, requesting participants to think about an action plan idea to be developed at the in-person training, answering final questions, and reviewing OWH-developed GFI. OWH shall ensure participants are provided OWH-developed GFI to include, but not limited to, resources list, learning opportunities, and education handouts. A list of

additional resources and websites provided as GFI may be accessed during training events depending on the participant's learning needs.

- 5.6.1.8. Ensure the physician and nurse instructor adhere to the training event schedule agenda as close as possible, maintaining time allotments for each discussion or activity. Attention to timekeeping and the holding of participants accountable to the agenda will be required. The training event shall last approximately 4-4.5 hours (inclusive of two (2) 15-minute break), start and end times will be determined by OWH and communicated to the contractor in advance of the training event.

Deliverables:

- Documentation of physician and nurse instructors mastering virtual content, completing virtual training practice session(s), and instructing and facilitating all content comprising the virtual training event, adhering to training schedule.
- Post Training Reports

6. PROFESSIONAL AND EXPERIENCE REQUIREMENTS

6.1. The contractor shall ensure physician and nurse instructors have the appropriate professional background and experience as listed in the table below. Resumes for all physician and nurse instructors clearly documenting stated requirements shall be provided by the Contractor to OWH for approval.

Physician Instructors (PI) and Nurse Instructors (NI)	
A minimum of three (3) years' experience providing clinical care to women in an outpatient setting. Clinical care experience must include but is not limited to, abnormal uterine bleeding and pelvic pain care, pre- and post-menopausal care, family planning and contraceptive management, menopause care, interpersonal violence, and care across the lifespan. Experience in Veteran-specific health care is preferred but not required.	
A minimum of one (1) year experience serving as an educator of healthcare professionals to include direct instruction in a small group or large group setting.	
<i>All required experience as described must have occurred after completion of professional training (including residencies, fellowships, and/or certifications).</i>	
Experience in excellent, clear and effective communication and interpersonal skills as demonstrated in past performance. This includes the ability to effectively communicate, listen to others, work collaboratively, function autonomously, adapt to changes in the work environment, respond to internal and external customer needs and expectations in a positive manner and problem solve. Essential job requirements include adherence to all relevant policies, procedures and guidelines affecting the work environment, as well as maintenance of required competencies.	
Be fluent in written and spoken English, be U.S. citizens or provide evidence of eligibility to work in the U.S. including an e-verify report. Physician and nurse instructors must be computer literate; skills teaching in a virtual environment preferred.	
Physician Instructors	Nurse Instructors
Hold either a Doctor of Medicine or Doctor of Osteopathic Medicine degree and hold a current, active medical license in any state and have completed an ACGME-accredited residency training program in family medicine, internal medicine, or obstetrics/ gynecology. Board-certification is highly preferred.	Hold at least a Bachelor of Science Nursing resulting in registered nurse designation (RN) but may have additional education such as master or doctoral level degrees or hold additional certifications and hold a current, active nursing license in any state.
Strong foundational knowledge in the pathophysiology and clinical management of women's health topics to include, but not limited to,	Strong foundational knowledge in nursing care (nursing assessment, triage, assistance, patient education, care

abnormal uterine bleeding, pelvic pain, vaginal discharge and sexually transmitted infections, breast and cervical cancer screening, breast and pelvic exams, contraception, and menopause care.	coordination) of women's health clinical topics to include, but not limited to, abnormal uterine bleeding, pelvic pain, vaginal discharge and sexually transmitted infections, breast and cervical cancer screening, supporting during breast and pelvic exams, contraception, and menopause care.
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6.2. Gynecological Teaching Associate (GTA)

The Contractor shall ensure the GTA has the appropriate physical attributes noted below and experience history. CVs for all GTAs shall be provided by the Contractor to OWH for approval.

The GTA must have, at a minimum, at least one (1) unaltered breast (piercing acceptable, prefer without augmentation although it does not make a difference in learning basic breast exam skills), a uterus (presence of intrauterine device acceptable), at least one (1) ovary, and who, at the time of the training event is not currently pregnant, not currently menstruating, and not currently using any vaginal device that obscures full view of the cervix such as, but not limited to, cervical cap, diaphragm or pessary. The GTA shall not have health issues which further spread a communicable disease through execution of the simulation scenario. The GTA must be able to provide up to four (4) complete breast and pelvic exams, to occur between the hours of 12:00 pm and 5:00 pm (local time) for each training event (timeframe also includes introduction, pre-brief and debrief activities described below).

The GTA must have at least one (1) year experience and training in using their own bodies as models to teach breast and pelvic examination without the assistance of faculty and providing immediate verbal feedback on examination technique in a supportive environment for skill development and practice. Experience in Veteran-specific health care is preferred, but not required (i.e., military sexual trauma).

The GTA must have clear and effective, communication and interpersonal skills as demonstrated by their experience. This includes the ability to effectively communicate, listen to others, work collaboratively, function autonomously, adapt to changes in the work environment, respond to internal and external customer needs and expectations in a positive manner and problem-solve. Essential job requirements include adherence to all relevant policies, procedures and guidelines affecting the work environment, as well as maintenance of required competencies. The GTA must be fluent in written and spoken English, be U.S. citizens or provide evidence of eligibility to work in the U.S. including an e-verify report.

6.3. Equipment Support Individual (ESI)

The Contractor shall ensure the ESI is computer literate, possess technical skills to support audiovisual equipment, has strong organizational skills to manage numerous pieces of Government Furnished Equipment (GFE), and is able to perform multiple tasks and ensure consistent operations within the training initiative. Knowledge of clinical simulation equipment and basic clinical simulation education principles is beneficial.

The ESI must have clear and effective communication and interpersonal skills. This includes the ability to effectively communicate, listen to others, work collaboratively, function autonomously, adapt to changes in the work environment, respond to internal and external customer needs and

expectations in a positive manner and problem-solve. This individual must be fluent in written and spoken English, be a U.S. citizen or provide evidence of eligibility to work in the U.S. including an e-verify report and possess a valid driver's license and able to drive a rented transport van (e.g., minivan or conversion van). The ESI must be able to lift moderately heavy objects (e.g., boxes and shipping cases up to 100 pounds in weight) on regular basis, including transferring in and out of a transport van and clinical sites on multiple occasions.

Deliverables:

- Instructors and ESI: Documentation of credentials, proof of experience as requested in this section.
- GTAs: Proof of experience, training, confirmation of personal physical attributes, and physical health assessment/examination documentation.

7. DELIVERY SCHEDULE

Section	Deliverable	Delivery Date	Submit to
5.1	Schedule "kick-off" meeting	Within 3 business days of award	COR & OWH
5.2	Meeting minutes, monthly progress, status, and management reports informing of the progress of each task, identifying accomplishment, issues, concerns, and proposed resolutions.	Monthly after the kick-off meeting	COR & OWH
Section	Deliverable	Delivery Date	Submit to
5.3	Documentation of viewing and mastering all content and attendance of face-to-face "train-the-trainer" training.	Within five (5) business days prior to 'train-the-trainer' workshop	COR & OWH
5.4	Documentation of instructing and facilitating all content (to include participant sign-in sheet) comprising the training event, adhering to training schedule.	Each training event	COR & OWH
5.6	Documentation of ESI receiving, transporting, managing, inventorying, and shipping GFE.		
5.5	Post-training report by instructors, GTA	Within five (5) business days after each training event	COR & OWH
	Post-training report and inventory list of GFE by ESI	Within five (5) business days after each training event	COR & OWH
	Documentation of attendance at post-training feedback sessions	Monthly during months of any training event	COR & OWH
6	Proof of experience and training documentation.	Upon Proposal delivery and with new staff hires	CO & COR

8. GENERAL REQUIREMENTS

8.1. Quality Assurance

The Contractor shall develop and maintain an effective quality control program in accordance with their approved Management Oversight Plan (MOP) to ensure services are performed in accordance with the PWS. The contractor shall develop and implement procedures to identify, prevent, and ensure non-recurrence of defective services. The contractor's quality control program is the means to ensure that the work complies with the requirement of the contract. As a minimum, the contractor shall develop quality control procedures that address the tasks and deliverables described in Sections 5 and 6 of the PWS. OWH will provide the Contractor with report templates used to evaluate performance.

8.2. Safety Protocols

Safety Protocols. The Contractor shall ensure all its physician and nurse instructors, GTAs, and ESIs follow VHA guidance and adhere to local VHA facility infection control policy while at in-person training events. This may include but is not limited to documentation of mandatory vaccination and/or testing (ex. influenza, coronavirus), non-attendance if symptomatic, social distancing measures, wearing a face mask, frequent use of hand sanitizer and/or hand washing, and increased cleansing and disinfecting of frequently touched items and equipment used during in-person training events. Current VHA guidance and local VHA facility infection control policy would be communicated by OWH in advance of the training event. Contractors shall provide their own infection/safety policy for their employees to OWH for awareness.

Staff Injuries. All injuries or illnesses suffered by contract employees in connection with the performance of this contract shall be the exclusive responsibility of the contractor. The contractor shall hold the Government harmless from, and indemnify it against, any liability arising from such injuries or illnesses.

8.3. Performance Requirement Summary (PRS)

The Government will evaluate the Contractor's performance as noted in the table below. This plan is primarily focused on what the Government must do to ensure that the contractor has performed in accordance with the performance standards.

Performance Indicator	Performance Standard	Acceptable Quality Level	Frequency	Method of Surveillance
Task 5.3	Contractor demonstrates mastery of all GFI and GFE each training event to include "train-the-trainer"	Score of at least 4 on a 5-point Likert scale on summative quantitative data sources	Each "train-the-trainer" workshop and each training event	Direct observation by OWH and/or learner evaluation
Tasks 5.4 & 5.6	Contractor demonstrates ability to effectively instruct, facilitate and lead all elements of training event Contractor demonstrates ability to manage all aspects of GFE onsite at training events including GFE transportation and security; assists and	Score of at least 4 on a 5-point Likert scale on summative quantitative data sources	Each training event	Direct observation by OWH, site POC and learner Evaluations

	supports instructors and VHA point of contact at each training site			
Tasks 5.5	Contractor will meet all deliverables dates as stated in the related tasks.	Provides completed reports within 5 business days of each training event 98% of the time	Within 5 days of each training event.	Random samples to ensure deadlines are being met

8.4. Government Furnished Information (GFI) and Government Furnished Equipment (GFE)

All content, equipment, and supplies to support the “train-the-trainer” workshop and all training events will be government-furnished and provided by OWH except for two (2) Contractor-furnished laptop computers, both with speaker and headphone capabilities, and include necessary connections, ports and/or adapters that allow connection to a larger projection screen (i.e., USB, HDMI). Contractor shall ensure proper usage of government-furnished equipment (GFE) to support the training event to include breast simulation equipment, pelvic simulation equipment, gynecologic items (“show-and-tell” items), consumable items for breast and pelvic exam activities, and ancillary items such as power cords and portable tables. Usage of GFE will be in accordance with manufacturer recommendations and follow OWH guidance. Primary responsibility of GFE once deployed to the field for training will be assumed by the equipment and set-up support individuals upon receipt and until return of said GFE at a designated location.

8.5. Security

8.5.1. Privacy Requirements

All content and materials used to support this training initiative are the property of the government and are only to be used during training events and for other purposes. All Contractors and Contractor personnel shall be subject to the same Federal laws, regulations, standards and VA policies as VA, and VA personnel, regarding information and information system security. There will be no VA sensitive information used or accessed. The Contractor shall provide two (2) laptop computers to display training materials during each training event; no connection to VA network will be necessary. The certification and accreditation of the contractor system (C&A) requirements do not apply. A Security Accreditation Package is not required.

8.5.2. Facility Access Requirements

Contractor personnel who require routine physical access to federally-controlled facilities must be issued a Personal Identity Verification (PIV) card in accordance with FIPS 201-3, OMB Memorandum M-05-24, and HSPD-12 requirements.

The Contracting Officer Representative (COR) designated PIV verifier will validate each contractor employee’s PIV credential prior to the start of contract work.

Contractor hiring shall include identity proofing, background investigation, and enrollment in the approved PIV issuance system.

Card Issuance and Return

PIV cards shall be obtained only from FIPS 201-approved sources, such as the agency or GSA/USAccess under the appropriate Federal Supply Schedule.

Contractor shall return PIV cards promptly when the card is no longer needed, when the individual leaves employment, or when the contract is completed or terminated. Failure to return issued PIV cards may result in delayed final payment.

Applicable Contract Clause

The Government will incorporate FAR 52.204-9, Personal Identity Verification of Contractor Personnel, into this contract, requiring compliance with all PIV credentialing procedures.

8.6. Confidentiality And Non-Disclosure

Except for uses and disclosures of VA information authorized by this Task Order for performance of the Task Order, the Contractor may use and disclose VA information only in two other situations: (i) in response to a qualifying order of a court of competent jurisdiction, or (ii) with VA's prior written approval. The Contractor will refer all requests for, demands for production of, or inquiries about, VA information and information systems to the VA Contracting Officer for response.

8.7. Agency Program Contact

Christine Kolehmainen, OWH, Director of Education; Bridget Souza, Project Manager, OWH; Karylle Ball, Management Analyst, OWH. E-mail: Christine.kolehmainen@va.gov, bridget.souza@va.gov, and Karylle.ball@va.gov.